

Raw Input:

Great. Let's analyze **Emergency Case - is it just emergency OT or other emergency charges too and emergency bed?**

The goal is to understand when the doctor writes emergency treatment A.

How should FC estimate builder handle that?

FC Estimate Builder would have knowledge of treatment A but how does that emergency change the billing or make additions to the billing? That is what needs to be understood so that logic can be integrated into the FC Estimate Builder.

Codex Output:

Emergency should be implemented as a billing overlay on Treatment A—not as a separate treatment and not as one universal “emergency surcharge.”

The most important distinction is that these are separate facts:

- Treatment A is clinically urgent/emergency.
- Patient passes through the Emergency Department.
- Patient occupies an emergency bed.
- Treatment/procedure happens during emergency OT hours.
- The admission is medico-legal.
- The payer agreement permits an emergency surcharge.

One does not automatically imply the others.

What the historical bills show

Among the 13,974 clean admissions:

Component	Admissions	Historical pattern
Emergency origin / ER physician	3,182	Every currently flagged emergency-origin admission has D000806 – ER Physician
ER initial assessment	1,786	Predominantly billed to insurance patients

Emergency bed	1,071	Separate 1–4 hour charge before/alongside the inpatient stay
MLC	403	Only 365 also had emergency origin; 38 MLC cases did not
Emergency OT codes	0	Codes exist in the tariff master, but none were found in clean historical service lines

Typical amounts

Service	Code	Historical median
ER Physician	D000806	₹1,000
ER Initial Assessment	EME5060	₹3,000
Emergency Bed, 1–4 hours	EME0065	₹1,310
MLC Charges – IP	HSP0047	₹1,200

These must ultimately use the resolved tariff/organization rate. For example, current emergency-bed rates range approximately from ₹1,100 to ₹1,970 depending on the tariff.

Emergency bed is a genuine additional component

All 1,071 admissions with an emergency-bed charge subsequently also had ward or ICU accommodation, averaging approximately 4.07 inpatient days.

Therefore:

- Emergency bed is not the room category for the entire admission.
- It represents ER holding/observation for 1–4 hours.
- Ward/ICU billing continues after transfer.
- It should only be included when ER-bed utilization is expected.

Its historical usage fell sharply after July 2025 even though the tariff code remains active. Therefore it should not be automatically added to every current emergency admission.

ER initial assessment is payer-sensitive

The initial-assessment line appeared in:

- 719 of 764 GIPSA emergency admissions: 94%
- 1,043 of 1,117 Non-GIPSA emergency admissions: 93%
- 21 of 1,267 Cash emergency admissions: 2%
- 3 of 34 Corporate emergency admissions: 9%

This is too strong a pattern to ignore. For the initial implementation:

- Insurance: default to included when the patient actually enters through ER, subject to tariff availability.
- Cash/Corporate: default to excluded unless explicitly selected or supported by the applicable tariff/agreement.
- Do not use one rule across all payer buckets.

Emergency OT

The hospital policy defines dedicated emergency OT codes:

- Normal OT: **OTC0005–OTC0020**
- Emergency OT: **OTC0054–OTC0069**
- Durations: ½ hour through 8 hours
- Emergency timing: Sundays, public holidays, and every day from 8 PM to 8 AM

The narrative policy also says holiday emergency charges are subject to the discretion of the unit head and anaesthetist. See [Billing Policy Details and Codes.json](/Users/reylvanttambi/Documents/New project 3/RawData/Billing Related Data/Billing Policy/Billing Policy Details and Codes.json) and [IP Billing Policy 2.0](/Users/reylvanttambi/Documents/New project 3/RawData/Billing Related Data/Billing Policy/IP Billing Policy 2.0 - Secbad.parse.md).

Critical implementation point: emergency OT must replace the corresponding normal OT line.

Emergency OT adjustment

= applicable OT-E tariff rate

– normal OT rate already included in Treatment A

Do not add the full emergency OT rate on top of a normal OT charge already present in the estimate.

The uplift is not a universal percentage. Across current tariffs, emergency-to-normal OT relationships vary materially. The Builder must look up the actual OT-E code using:

tariff + organization + room category + OT duration

Historical bills contain normal OT codes but no emergency OT codes, including cases where FC remarks contain “emergency.” Therefore the emergency OT policy is tariff-backed, but its actual historical execution is not yet independently validated from the clean service-line data.

Agreement-specific package surcharges

Some insurance agreements have a separate emergency percentage rule:

- Bajaj: 15% extra on listed packages for public-holiday/Sunday admissions.
- ICICI: 10% against eligible procedures performed between 8 PM and 8 AM, on Sundays, or public holidays, with documentary justification.

See [Bajaj package guidelines](/Users/reyvanttambi/Documents/New project 3/RawData/packages/Insurance Manually Cleaned Pkgs/BAJA_HYD_23_clean_package_guidelines.json) and [ICICI package guidelines](/Users/reyvanttambi/Documents/New project 3/RawData/packages/Insurance Manually Cleaned Pkgs/SSG_ICICI_clean_package_guidelines.json).

These rules must be organization/agreement-specific. They are not general Non-GIPSA or GIPSA rules.

For a package case:

1. Check for an explicit package emergency clause.
2. Apply the stated percentage to the specified package/procedure base.
3. Do not also add an OT-E uplift for the same event unless the agreement explicitly allows both.
4. ER physician, ER bed or MLC can still remain separate where allowed/excluded by the package.

MLC is independent

MLC must never be inferred merely because a case is emergency.

The Builder should ask separately:

Medico-legal case? Yes / No

If yes:

- Add one tariff-backed **HSP0047** charge.
- For insurance, classify it as a patient-payable/NME component.
- Preserve it separately from other emergency adjustments.

Other emergency services

Emergency admissions also contain items such as:

- Suturing
- Intubation
- CPR
- Oxygen
- Nebulization
- Transfusion
- Catheterization
- Emergency investigations
- Emergency-specific procedures such as emergency endoscopy or embolisation

These are actual-use clinical services, not automatic consequences of an emergency flag.

Where the tariff master contains a distinct emergency version of Treatment A, the Builder should use that specific service/procedure code. Otherwise these services should be estimated from matched emergency-treatment history and presented as a variable range—not inserted as a standard emergency surcharge.

There is no evidence supporting a universal emergency uplift to surgeon, anaesthetist or other professional fees.

Recommended Builder flow

When the doctor writes “Emergency Treatment A,” the Builder should retain Treatment A and open an Emergency section with these inputs:

1. Did the patient enter through Emergency?
2. Is an emergency bed expected?
3. Expected ER-bed hours—currently supported only for 1–4 hours.
4. Does Treatment A require OT/Cath Lab?
5. Expected procedure date and start time.
6. Is that time 8 PM–8 AM, Sunday or a public holiday?
7. Is it an MLC case?
8. Is this package, open billing or package-plus-open?
9. Does the payer agreement contain an emergency clause?

The estimate should then be:

Treatment A emergency-neutral estimate

+ ER physician

+ applicable ER initial assessment

- + emergency bed, if actually expected
- + emergency procedure adjustment
 - exact emergency procedure code, or
 - OT-E minus normal OT, or
 - package/MOU emergency percentage
- + MLC, if applicable
- + estimated variable emergency-care services

Only one procedure-level emergency pricing method should ordinarily apply.

Important data-model correction

The existing `has_emergency_origin` field is retrospective billing evidence—it is effectively anchored to the ER Physician line. It is not sufficient to represent what the doctor wrote.

The Builder should maintain separate structured fields:

`is_clinically_emergency`
`arrived_via_emergency_department`
`emergency_bed_expected`
`emergency_bed_hours`
`procedure_expected_at`
`emergency_ot_eligible`
`emergency_ot_approval_status`
`is_mlc`
`emergency_pricing_method`
`emergency_rule_id`

Free-text parsing should only create a suggestion. In 294 FC records containing the word “emergency,” only 66 had historical ER-origin evidence. Some mentions were emergency phone numbers or generic counselling language. Therefore the word itself must not trigger charges automatically.

The correct product behaviour is: “Emergency Treatment A” activates the emergency decision workflow; it does not immediately add every emergency charge.